

Metro Referral Unit

Tel: 1300 110 600

Fax: 1300 546 104

30/03/2026

Calvary

P.O. Box 7310

ADELAIDE, 5000

Tel: 1300 797 522

Fax: (08) 81791581

REFERRAL

Patient: Ms Helen Burke

Patient ID: 329000

MRN: 010634000

DOB: 18/06/1946

Sex: F

Medicare No: 5104683000 - 1

Phone: 0423 001 777

Address: 50 Reisling Avenue, GLENGOWRIE, SA, 5044

Gen. Practitioner: Glengowrie Medical
Centre

Dear Calvary

Please see confidential information from Metropolitan Referral Unit.

Contact the unit on 1300 110 600 with queries you may have with the attached information.

Liaison Name: Alanna Ortmann

Pages:

9



URGENT

Referrer :

MRU ()

Metro Referral Unit

Phone:

Signature

Date : 30/03/2026

Referral Unit

Patient: BURKE, Helen

Date: 30/03/2026

Patient ID: 329000

DOB: 18/06/1946

MRN: 010634000

Sex: Female

Age: 79 yrs

Mobile: 0423 001 999

Medicare No.: 5104683000

Indigenous Status: Neither Aboriginal or TSI
Origin

Gen. Practitioner: Glengowrie Medical
Centre

Living Arrangements: Lives With Spouse

Country of Birth: Australia

Address Category: (Primary) (Mailing) (Invoicing)

Address: 50 Reisling Avenue
GLENGOWRIE SA 5044

Next of Kin: Graham Browne(husband)

NOK Mobile: 0438 632 894

Referral Source

Referring Source: Comm – SAPS

HA \ SD: ED Avoidance

Referral Date: 30/03/2026

Referral Name: G. Georgopoulos

Allergies: Drug Allergy - Nil Known

ICC Name: MRU

Contact NO.: 0479188047

ICC Contact No.: 1300110600

Position: Nurse – NUM/NC

Known Hazards/Alerts

Known Hazards: has a cat

Primary Diagnosis

Diagnosis: Biliary carcinoma

Past History: L) thalamic ischemic stroke - no residual, gallbladder malignancy, Gall bladder perforation, Intra + peri hepatic collection
in setting of malignancy of gallbladder - drain fell out, not being replaced

Management Plan / Care Requested

Management Plan: Helen is a 79 year old living at home.

Referred by SAPS for palliative care and wound care.

Commencement date: 30/3/26 ASAP

Frequency: twice weekly

Care: palliative care and wound care. (wound care can be completed during pall care visits but some C visits provided too in case patient needs additional wound care between palliative visits).

AKPS = 50

RUG = 8

Phase = 3

Prognosis measured in weeks

SYMPTOMS/CARE NEEDS:

- 1) provide support and education for patient and family
- 2) Wound care: liver drain for ascites fell out, patient does not want drain back in, now leaking +++, RUQ area, under R) breast, please assess and dress appropriately on 30/3/26 ASAP
- 3) symptom assessment and monitoring

Attachments:

Other information: nil

Package Information

Program 1: SA Comm Care

Episode Total Visits: 20

Provider	Matrix	Visits	Service Commencement date
Calvary	RNP - (12)C - (8)	20	30/03/2026

MRN: 01063400	BURKE, HELEN BARBARA (MS)	Flinders Medical Centre
Visit: 125324741		Current Location: FMC-OP
Age: 77y (	Gender: Female	Flats D1

Palliative Care Note-- PallCare (FMC) [Charted Location: FMC-OP Flats D1] [Date of Service: 30-Mar-2026 13:57, Authored: 30-Mar-2026 13:57]- for Visit: 125324741, Complete, Entered, Signed in Full, General

Palliative Care Note

Situation

History of Presenting Issue: Southern Adelaide Palliative Services – Community Phone Call

3 Patient identifiers confirmed:

Patient Profile: 77y.o lady from home with husband Graham in 2-storey home.
Have registered with MAC this week, awaiting assessment.
They have a shower chair as the only equipment.
Calvary Care for biliary drain dressing.

Diagnosis: Biliary carcinoma – 11/2025

Extent of disease/Treatment to date:

Gallbladder perforation in the context of likely unresectable gallbladder cancer complicated by large subcapsular intrahepatic collection. Percutaneous drainage performed 24/12/25 - nil growth on MCS apart from scanty growth *C. albicans*.
ERCP performed 29/12/25 showing patent stents without overt leaks on cholangiogram.
Repeat CTAP (09/01): Slight reduction in size of the peri-hepatic collection compared to 29/12/2025. The middle and right hepatic veins are not as well opacified compared to 29/12/2025. Whilst no direct visualisation of thrombus, vein patency could be confirmed with ultrasound if clinically indicated. No other interval changes in the abdomen.
Awaiting oncology appointment, not 100% sure she wants to go through ongoing treatment.

GOC: 26/02/2026; Helen is not for CPR, intubation or ICU treatments. Helen wishes to focus on treatments to maximise her comfort and quality of life. She would still consider simple life prolonging treatments. She wishes to avoid ED and hospital as much as possible. If she requires admission she would like to consider private hospital or hospice if possible.

Medications:

amoxicillin clavulonic acid 875/125 BD (preventative ongoing)
fluconazole 200mg mane
probiotics
Vitamin C
Vitamin D
Asprin 100mg man

Requested by: Georgouloupoulos, Gina (Registered Nurse), 30-Mar-2026 14:08	Page 1 of 3
--	-------------

	(MS) Gender: Female	Current Location: FMC-OP Flats D1
--	------------------------	--------------------------------------

Panadol Osteo BD
Temazepam 10mg prn
oxycodone 5mg prn (can use Q2H as per Dr Emily)
ondansetron 4mg s/l prn
metocloperamide 10mg TDS prn NEW
coloxyl & senna prn
Buprenorphine patch 5 microg/hour, NEW placed on Thursday night

Call from: Helen, client.

Reason For Call: Has some bad news.

The collection is leaking and swelling around the site. Bright yellow ooze.
Came out with a gush.
Only weeping a bit at the moment.
Does not want to return to a drain,
Not causing any pain.

Noted 3/03/2026 from ID- not keen for further drain insertion or surgical intervention or oncological intervention.

Calvary Care managing drain, would like to try Silverchain.

Currently covered with a large bandaid with a pad thing.

RUQ- frontal plain, under right breast.

Plan: MRU referral for dressing changes.

[Faint, illegible text, likely bleed-through from the reverse side of the page]

(MS)
Gender: Female

Flinders Medical Centre
Current Location: FMC-OP
Flats D1

Electronic Signature
Georgouloupoulos, Gina (Registered Nurse) (Signed 30-Mar-2026 14:08)
Authored: Situation, Assessment, Letter, Send Summary To, AH Data

Last Updated: 30-Mar-2026 14:08 by Georgouloupoulos, Gina (Registered Nurse)

(MS)
r f e m

Current Location: FMC-OP

Infectious Disease Clinic Note-- (FMC) [Charted Location: FMC-OP L2 Medicine] [Date of Service: 03-Mar-2026 12:50, Authored: 03-Mar-2026 12:50]- for Visit: 125684041, Complete, Entered, Signed In Full, General

REASON FOR VISIT:

- Reason for Visit [REDACTED] phone review

ALLERGIES:

No Known Allergies: Active

PROBLEM LIST:**Visit Reason:**

- Phone Call: Status: Active, Scope: Visit, Onset Date: 26-02-2026, Description: Phone Call

Past Surg Hx:

- Cholangiogram, biliary duct dilatation, biliary stents - Nov 2025: Status: Active, Scope: General, Onset Date: 24-11-2025, Description: (FMC) Consultant: Steve Chrysosidis

CLINIC SUMMARY:**CLINIC SUMMARY:**

• Summary of Appointment

Infectious Diseases RMO Phone Call Review - Shanas
Consultant: Dr Teddy Teo

Profile:

77-year-old female from home with husband

Background: Left thalamic ischaemic stroke (2021), Localised anterior gallbladder perforation with intrahepatic mass (malignancy of unknown primary)

ID Issue

Gallbladder perforation in the context of likely unresectable gallbladder cancer complicated by large subcapsular intrahepatic collection

- Admitted after being referred from medical oncology OPD appointment with CT scan findings raising concern for gallbladder perforation with a large subcapsular intrahepatic collection. Had increasing abdominal pain and reduced oral intake leading up to admission
- Percutaneous drainage performed 24/12/25 - nil growth on MCS apart from scanty growth *C. albicans*
- ERCP performed 29/12/25 showing patent stents without overt leaks on cholangiogram
- On IV piperacillin/tazobactam and PO fluconazole
- Repeat CTAP (09/01): Slight reduction in size of the peri-hepatic collection compared to 29/12/2025. The middle and right hepatic veins are not as well opacified compared to 29/12/2025. Whilst no direct visualisation of thrombus, vein patency could be confirmed with ultrasound if clinically indicated. No other interval change in the abdomen.
- USS abdomen 27/1/26: no thrombus
- Seen oncologist Dr Amitesh Roy 27/1/26 in FPH, opted for palliative treatment, declined any further treatment
- Repeat CT abd/pelvis (Benson) 13/2/26: Generally similar size of primary tumour. Largely unchanged

Requested by: Georgouloupoulos, Gina (Registered Nurse), 30-Mar-2026 14:16

Page 1 of 4

(MS)
Gender: Female

Flinders Medical Centre
Current Location: FMC-OP
L2 Medicine

subcapsular collection of the liver with drain in situ. Small right pleural effusion.

- Pigtail dislodged on 15/2/26, Helen declined further pigtail insertion, seen in Hep bil clinic on 18/2/26, Helen still declining drain reinsertion, hence given PRN Hepbil clinic appointment

- Completed 4/52 course of IV piperacillin-tazobactam, planned for further 4/52 of PO Augmentin

R) pleural effusion

- MET call 29/12 for tachypnea

- CTPA (29/12): Moderate right sided pleural effusion with associated collapse. There are subtle ground glass changes seen within the right lower lobe also noted which may be in keeping with an early infective/inflammatory process.

- Bedside USS (30/12): Right: small, simple, anechoic effusion, measuring 2-3 rib spaces, 2cm in depth, with additional subpulmonic component. Perihepatic fluid also noted.

- Diagnostic aspirate (31/12): pH 7.47. Exudate. Culture nil growth. No malignant cells on cytology.

Microbiology:

- Blood cultures

- 29/12 - nil growth

- Fluid MCS

- 24/12 - +poly, NBS, scanty growth Candida albicans

- Respiratory NAT swab

- 28/12 - negative

Antimicrobials:

- IV metronidazole 500mg 12-hourly (23/12 - 28/12)

- IV ceftriaxone 2g 24-hourly (23/12 - 28/12)

- IV piperacillin/tazobactam 4.5g 6-hourly (28/12 - 25/01)

- PO Augmentin 875/125 mg BD (26/01 - current)

- P/O Fluconazole 200mg daily (31/12/25- ongoing)

Phone call to palliative care consultant,

Helen has been maintaining well, has managed to spend quality time with family, has declined further surgical intervention

Phone call to Helen,

has been feeling fine since discharge, no fever, not in pain, no issues since drain fell off discussed about ct abdomen/pelvis findings done in Benson on 13/2/26, no changes in subcapsular liver collection

Informed Helen, infection is currently being suppressed by oral antibiotics, however without source control, there is high chances that antimicrobial therapy may fail in the coming future discussed about cessation of oral antibiotics vs to continue oral antibiotics for the time being given Helen is maintaining okay at home

Helen is keen to continue oral antibiotics for now, hasnt had any adverse events to antibiotics

Informed Helen, I will give her a new script for 2 months and book her into Dr teddy Teo's clinic in 2 months time for a face to face review in which Helen was agreeable to

However, reemphasized to Helen that given source control not achieved, there is risk of antimicrobial failure even with oral antibiotics and in the event this occurs, in keeping with Helen's goals of care wishes, it would be best to cease antibiotics and focus on comfort measures

Requested by: Georgouloupoulos, Gina (Registered Nurse), 30-Mar-2026 14:16

Page 2 of 4

	(MS) Gender: Female	Current Location: FMC-OP L2 Medicine
--	------------------------	---

Helen understanding of this

Assessment

- # Gallbladder perforation in the context of likely unresectable gallbladder cancer complicated by large subcapsular intrahepatic collection
- repeat Ct abd/pelvis on 13/2/26: showed unchanged subcapsular collection of the liver
 - not keen for further drain insertion or surgical intervention or oncological intervention
 - known to palliative care
 - however, keen to continue oral antibiotics and antifungals as she has been clinically stable

Plan

Continue PO amoxicillin/clavulanic acid 875mg/125mg BD and PO fluconazole 200mg daily (scripts faxed to Glenelg East terry White Chemist)
OPD clinic with Dr Teddy Teo in 8 weeks time (face to face review)

Shanas

Infectious Diseases RMO
HPAT Phone - 0403 926 852

- Follow Up Plan
- Send letter to:
- Printer Address

Discharge
No letter required
\\hit433vprt001\FMC-Printer-1148

SEND SUMMARY TO:

GP Details:

- | | |
|---------------------------|---------------------------|
| • Title | DR |
| • GP Surname | JACOBSON |
| • First Name | COLIN |
| • Practice Name | GLENGOWRIE MEDICAL CENTRE |
| • Postal Address | 144 MORPHETT ROAD |
| • Postal Suburb, Postcode | GLENGOWRIE, 5044 |
| • Postal State | SA |
| • Street Address | 144 MORPHETT ROAD |
| • Suburb, Postcode | GLENGOWRIE, 5044 |
| • State | SA |
| • Phone Number | 08 8166 3630 |
| • Fax Number | 08 8166 3631 |
| • Provider Number | 0474326L |

Electronic Signatures:

Binti Shahul Hamid, Siti Shanas (Medical Officer) (Signed 03-Mar-2026 13:34)

Authored: REASON FOR VISIT, ALLERGIES, PROBLEM LIST, CLINIC SUMMARY, SEND SUMMARY TO



Last Updated: 03-Mar-2026 13:34 by Binti Shahul Hamid, Siti Shanas (Medical Officer)